[00:00:03] David Kaufer: Welcome to the lighter side of the spectrum. The podcast that celebrates the multiple hats we wear and the juggling acts we perform in life. Whether you're parenting kids and teens on the autism spectrum, navigating the ever-evolving healthcare industry, or building a more inclusive and impactful world, this is your space to find connection, inspiration, and growth. Together, we'll share stories, tackle challenges, and explore how innovation and inclusion create better outcomes for everyone. Because no matter the spectrum, there's always a lighter side. Now here's your host. David Coffer. Hello again, everybody. This is David Coffer. Welcome back to the lighter side of the spectrum, where we explore the people, ideas, and stone. Hi. How are you? Good. Happy. Okay, no good. And conversations that help us better understand autism, communication, and what it really means to support human potential. You know, one of the things I've certainly learned over the last few years is that so many of the assumptions that we make about people turn out to be wrong. We've learned that what looks like a lack of understanding may actually be a motor planning challenge. We've also learned that what looks like a behavioral issue may be sensory overload. What appears to be an inability to communicate may simply be a person who hasn't been given the right way to communicate. Today's conversation is going to ask us to consider another assumption. What if vision isn't just about seeing clearly? You know, most of us think of vision as reading, the eye chart at your doctor's office, trying to see if you have 2020 vision. You know, if you're good, you're good to go. But what if your eyes are healthy yet your brain isn't using them together in the way it's meant to be? What if the issue isn't how well you see, but how you see? And that's the fascinating world we're gonna step into today. And my guest today is Denise Allen, host of Healing Our Sight Podcast, and a passionate advocate for helping people better understand binocular vision and the remarkable adaptability of the human brain. Denise spent most of her life with Strabomide Strabimus. I knew I was gonna screw it up. Well help me, Denise. Strabismus. I even practiced it. And without stereoscopic or 3D vision, like many people, she assumed that was simply the way her brain worked. But as an adult, she discovered that our visual system may be far more adaptable than many of us realize.

[00:02:34] Barry Prizant: business.

[00:02:53] David Kaufer: And that journey led her into the worlds of vision therapy, neuroplasty, developmental optometry, and neuroscience. Along the way, she's had conversations with leading experts while helping parents, patients, and professionals better understand conditions and related to vision-related challenges. So this isn't about Autism specifically, but we certainly see it within autism. And I think it's really gonna be a fascinating conversation because vision, sensory processing, motor planning, communication, they're all parts of an incredibly complex system. So Denise, with all that being said, thanks for joining me today. I'm really looking forward to this conversation.

[00:03:36] Barry Prizant: Thanks for having me.

[00:03:38] David Kaufer: Yeah. So why don't you tell us a little bit about your experience? What tell us about what is it like to grow up with this condition?

[00:03:49] Barry Prizant: Well, you know, I didn't know anything different. So I thought I was pretty normal. But I did have struggles with coordination, being very clumsy, not very good at catching or th you know, balls or anything, any moving object coming at me was difficult to gauge where it was and catch it. So I was really bad at ball games. So I was usually chosen last, you know, that kind of thing.

[00:04:17] David Kaufer: Right.

[00:04:18] Barry Prizant: that posed a problem, of course. my knees were always scraped up, you know. I fell down. I just thought I was a clumsy kid, you know.

[00:04:30] David Kaufer: Yeah so you just thought you were clumsy, I mean right? Like you hear about that.

[00:04:34] Barry Prizant: So my mom put me in dance class. That was supposed to help. Didn't love it that much. No, it did it did help. And then I actually did gymnastics in junior high, which I shouldn't have probably been able to do very well. I was too tall for it. Wow. But yeah, that that's challenging. It's it doesn't stop someone from doing something that they want to do. My brother, who had the same condition, only his

[00:04:42] David Kaufer: No.

[00:05:04] Barry Prizant: showed up when he was a baby. he became a basketball player. He played college basketball, which also shouldn't really be very easy. But he was passionate about basketball. Wow. So he overcame that. So I think we don't have to discount anything, right? But the thing that I found is that it can be

[00:05:27] David Kaufer: Stone's very excited today. So again, he's making lots of appearances. He's getting ready to head out with his brother. That's amazing though. I mean, that he was able to play basketball. Because again, why do you I mean we'll back up a little bit and maybe kind of talk about Strabismus. Is that I get better that okay. what is it, you know, and in

[00:05:47] Barry Prizant: Yeah.

[00:05:53] David Kaufer: How is it defined and how does it impact people?

[00:05:57] Barry Prizant: Yeah, so strabismus is when the eyes don't focus together. And the easy explanation is just that they're fighting with each other all the time. And i in order to compensate for that, one of the eyes will shut off. And when you look at someone, often that eye is pointing in the another direction. Okay. Because it's getting out of the way of the other eye that's the strong eye. So you'll see. kids with an eye turn, right? Crossed eyes, w however you want to term it, right? It can be turning in or it can be turning out. people will call it a lazy eye.

[00:06:38] David Kaufer: Yeah, you hear you hear about that. Okay, so that's kind of

[00:06:43] Barry Prizant: And so it it's like five percent of the population that have an actual strabismus like this. The normal population. If you're looking at the autistic spectrum, then it's to totally different. And that's just strabismus. That's not other vision problems like inability to track or other focusing types of issues like that. You know, so my Mine is kind of a subset, but it's a much bigger problem than that.

[00:07:14] David Kaufer: Is this something that we're just like so many other things, we're just learning more about? I mean, is there just is it one that's we're just getting more awareness? Because growing up, I mean, what was like you said, you were just kind of considered maybe a little clumsy or you went in for traditional eye exams, right? Like, you know, was it just what did the eye doctors see or say, you know, and what did it take for you to finally figure it out?

[00:07:41] Barry Prizant: Yeah, that is a great question. And I and I think that it's always been a problem. I just feel like the fact that we can do something about it hasn't been very obvious. So when I my brother, it was obvious at birth. He went into the doctor, they did surgery on his eyes because it was pointing way in, you know.

[00:08:02] David Kaufer: So it was so obvious that like they're like, Okay, we gotta fix this. I mean

[00:08:06] Barry Prizant: Exactly. Yeah. And the traditional solution f when you have this kind of an eye turn is the ophthalmologist will say, Well, we're gonna do surgery. They'll do two or three surgeries. It often it takes more than one. My brother had three surgeries. Yeah, two when he was a baby and one when he was like seven to kind of tune it up. So it's kind of a lot of surgery for a young child. It doesn't teach the eyes how to work together though, to have surgery.

[00:08:20] David Kaufer: Oh wow.

[00:08:35] Barry Prizant: So even though my brother's eyes were fairly straight, he didn't learn how to see depth. My problem didn't surface until I was almost three. So it's a different type of strabismus. There's infantile like my brother, and then there's c a compensatory type of strabismus. And it usually means that you have either far sighted or nearsightedness and your eyes are not working together and they're compensating. And they put glasses on the child like they did me. And the eyes look pretty straight and the doctor says, Okay, you're good to go.

[00:09:09] David Kaufer: Just because just because your eyes are straight. Exactly.

[00:09:12] Barry Prizant: Exactly. And a I remember a as a child going in for my yearly exam and I was about ten and they did this test where you look in the big machine and on one side there's a note and on the other side there's the five lines for the music staff. And they said, Can you tell us what line or space that note's on? And I said, Well, I see the notes over here and I see the lines over here. Do you want me to guess? And I had been guessing until then. Something just clicked and I thought and I was and I was annoyed because we I'd had a traumatic event on the way to the office. And so I think I was pr probably a little more sassy than usual, you know?

[00:09:45] David Kaufer: Really? Yeah. A little edgy. A little edgy ten year old.

[00:10:02] Barry Prizant: And I just said, Do you want me to guess? Because I can't I see 'em on, you know, with each eye. And I'd had never been able to see through binoculars or, you know, the little kaleidoscope thing. The Yeah. I could never do those. and they just said, Oh, you can't do that. We won't do that anymore.

[00:10:15] David Kaufer: Oh yeah. Those were fun. Yeah. That was their answer? Yeah. Oh great. That's helpful. Yeah.

[00:10:30] Barry Prizant: And you know, vision therapy has been around that long but nobody knew about it. I didn't know about it until I went f looking for answers as a you know, forty something year old because my eyes were bothering me more at that point.

[00:10:46] David Kaufer: Well, I think there's still a real lack of understanding about vision therapy. yeah, it was something I'd heard about again in our journey with autism ourselves. That's just one of many avenues that you explore when you're dealing with communication. And I remember hearing about we have a place I'm in the Seattle area, and you know, checking out, but that was all new, just the whole idea of doing vision therapy, because again, it's so

[00:10:51] Barry Prizant: Absolutely.

[00:11:16] David Kaufer: yeah, you just think about just you just get eye exams and you get classes. You don't think about this as something another area, although it makes perfect sense once you start looking at it a little more closely.

[00:11:27] Barry Prizant: Right. Yeah. And I didn't start looking until my eye turn started to become really bad in my forties. I started to get presbyopia like a lot of us do at that age, you know. Can't see till you're, you know, have your hand clear out here. And so my eye turn just got worse and I looked at natural vision improvement things and I started searching online and I just by accident

[00:11:44] David Kaufer: Wow.

[00:11:56] Barry Prizant: Not by accident, right. Came across vision therapy, found out there was a doctor just down the street from me. And there were only two in the whole state of Utah at that point. And went in and had an exam and f figured out that there was something we could do.

[00:12:12] David Kaufer: Wow. Well that's I'm glad that you found that. What was it like then? I mean, what and how tell me about that experience as you after you went through started going through the therapy, did you experience like true depth perception for the first time or you know, things like that?

[00:12:29] Barry Prizant: Well, it wasn't quite that easy. Yeah. No. it was about two thousand thirteen. I started vision therapy, did a lot of sessions of therapy, and I just wasn't getting any solu any resolution at all. I wasn't getting any progress. And my doctor said, you know, you can look at surgery, that could be

[00:12:32] David Kaufer: It wasn't like

[00:12:59] Barry Prizant: What you might want to do in addition, and I didn't want to do surgery, so I stopped vision therapy, took my daughter in, found out that she was mini me and that she

[00:13:08] David Kaufer: Okay. It's really genetic, it sounds like

[00:13:10] Barry Prizant: Well, I knew Yeah. It's hard to say. It's yeah, it's hard to say. But I mean I have six children. My youngest daughter was just like me. I knew she was like me because she had an eye turn when she was three, like I did. And I had taken her to the doctor and they put her in glasses. It was just, you know, the same process. And she was fine. She did fine in school. She didn't seem to struggle that much in other things. She didn't seem to be s super clumsy. I had done other things with her when she was very young, like music and movement. I taught music and movement classes. And so there were things that maybe compensated better. I don't know. And I didn't see her doing poorly in school or having any struggles. And I had this passion for sharing the whole thing, you know, because I knew it could help. I hadn't helped me yet, but I knew it could. And so I was reading this book called W if when your ch Sorry When Your Child Struggles by David Cook. And it had a checklist in it of things to ask the child. And I was thinking about it in relationship to c children that I was experiencing at school when I was substitute teaching. But I practice I decided to practice the questions on my daughter. And so I said I said to her, Do the words ever do this when you're reading? And you know, the split apart. And she said yes. And I thought, Oh, that's not good. I had no idea that she was seeing double when she

[00:14:51] David Kaufer: And how old was she at this point? Remind me. Okay, so about okay. Yeah.

[00:14:53] Barry Prizant: She was nine. Okay. Yeah. So very m starting to be very aware of that. And potentially have problems later on, you know, because reading just the words just keep getting smaller, right? And more difficult. so I said, Well do you ever close an eye when you're reading? That was also on the checklist. And she said, Oh yeah, ev all the time when I'm reading in bed, I s I s I lay with my eye

[00:15:00] David Kaufer: Yeah.

[00:15:22] Barry Prizant: on the pillow so that I'm reading just with the one eye and she said and then I turn over when I turn the page and do it with the other eye. That was helping avoid double vision, right? And I had

[00:15:37] David Kaufer: She kind of s figured it out. Like she had her own solution. I mean to exactly cope with it was a co it was a coping mechanism, basically.

[00:15:45] Barry Prizant: Exactly. Yes. And I don't know if she did any of that at school. Sometimes children will, you know, they'll sit with their hand up like this, you know, cover an eye. And just make other compensator compensatory items that will help them to not have that problem. So she was doing all of those things. I had no idea. And I thought I was a pretty aware mom, you know, she was child number six. You know, it I kind of was beating myself up a little bit about it, you know. So I took her into my vision therapy doctor and he got her in vision therapy. It took about nine months of therapy for her and she was able to gain three D. So it was a pretty easy process. It is something for a lot of people that you kinda have to keep doing a little bit of to keep the skill.

[00:16:44] David Kaufer: Kinda like exercise in a way.

[00:16:46] Barry Prizant: A little bit, yeah. So I think she might have lost a little ground. We've talked about that over the years. She's been on my podcast too and told her story. So it she recognizes that there might be some things she could do to kind of strengthen her ability still. in the meantime my daughter

[00:17:02] David Kaufer: Yeah.

[00:17:04] Barry Prizant: It does. Yeah, it does. And for some people it takes way longer, you know? And there are cases that take less time. But I think nine months is a fairly standard or comfortable amount of time that it will take for people if they're if for kids, you know, kids are much more malle malleable than adults are. So oh

[00:17:25] David Kaufer: Yeah, absolutely.

[00:17:28] Barry Prizant: I went back in at the end of that process. My doctor had done some training in and some other kinds of therapy. I went in, I tried those, and still wasn't getting the results I wanted. At this point, I'm like 80 sessions in division therapy. And once again, he said, You probably need to look at the surgery. So He referred me to a doctor that he's worked with. I went in, I had the strabismus surgery. Okay. So that your eyes are lined up instead of crossed.

[00:18:09] David Kaufer: Wow. I mean that's a big step. I could see why you know, you would want to avoid it if at all possible. I think anytime for most of us, right, you talk about our eyes or eye surgery is just it's different. It hits different than just saying, Oh, you know, you we gotta do some elbow surgery, for example, or something like that.

[00:18:29] Barry Prizant: Well, I mean when you're cutting a muscle, I j I think it's y you tend to hesitate before you're

[00:18:36] David Kaufer: It's your eyes. It's your eyes. All right. Like that's very personal to all of us. So but it sounds like it was it sounds like obviously it was a success.

[00:18:47] Barry Prizant: Yeah, it was amazing. I had a really easy time with the surgery. I took like one pain med. I decided the pain meds weren't really helping me. I not doing good things to my body. Okay not doing that. And so I'm really into natural remedies. So I took this homeopathic remedy called Arnica. Oh, okay. Instead of any of the other drugs they had given me and I went back for my check up a m a week later and my eye doctor said, I can't believe how fast you've recovered from this surgery. Great, thank you. and I r that week I also went back into vision therapy. And they don't necessarily recommend that you do that. You know, a lot of times doctors will say, Oh, you should recover first or whatever. I mean I was recovered, but not everybody will go back in that quickly. And

[00:19:23] David Kaufer: Wow.

[00:19:43] Barry Prizant: As soon as I went back in, I could do everything that I hadn't been able to be for.

[00:19:50] David Kaufer: Wow. That must have been amazing.

[00:19:53] Barry Prizant: It was. Yeah. And I gained r you know, pretty good depth perception, especially close up. My depth perceptions in normal range. Further out, it's not quite as good. So I'm still working on that.

[00:20:06] David Kaufer: And I'm just curious, do you have twenty vision as well now? Or do you

[00:20:11] Barry Prizant: there's no relationship between the way that the eyes focus together and the acuity piece. Yeah. We think that vision is just all about seeing 2020. And it's not at all. So vision therapy doesn't fix that piece. Okay. I still need glasses. Okay. So I mean I wear contacts. But yeah, that piece doesn't get fixed by therapy.

[00:20:13] David Kaufer: Right. So the vision.

[00:20:40] Barry Prizant: like that. And there are other things that we can do to improve the acuity piece, but that's not typically done in a regular doctor's office. You know, that kind of goes into the realm of natural vision improvement.

[00:20:55] David Kaufer: Okay. But that's remarkable that you recover that quickly. Do you attribute it to the homeopathic approach that you took or just your you know, were there other steps that you took that maybe helped accelerate it?

[00:21:09] Barry Prizant: I think that that's what I've typically attributed it to, but I also am really healthy and mindset also plays into it. The fact that I had never had a surgery before made it easier. You know, people who've had other strabismus surgeries, there's scar tissue that's gonna get in the way and it makes it a harder process.

[00:21:33] David Kaufer: No, that makes a lot of sense. You know, early w you were talking about how it affects what you know, roughly five percent of the general population, but the pi the percentage is a lot higher when you're talking about the neurodiverse or the aut you know, autistic population. Do you have any estimates of how it is?

[00:21:53] Barry Prizant: I thought you might ask me that. So I pulled out this book that I like. It's called I can you see it?

[00:21:59] David Kaufer: Yeah, I guess yeah.

[00:22:00] Barry Prizant: The Hidden Link Between Vision and Learning by Wendy Rosen. She's one of my podcast guests. And I knew she had told me this and that I had read it. And so I looked it up. And it says, according to her, that about 25% of children in the in the United States struggle with learning problems. And then 25% of children also have a vision problem that impedes learning. So let's see, maybe there's a connection there, right? And according to the National Center for Health Statistics, 20 to 25% of children enter school with significant vision problems. So, and that's, I mean, that's not just strubismus, that's other vision issues, right? the greatest difficulty among learning disabled children is with reading, which is affecting 75% of that population. And 80% of these children struggle with one or more visual skills.

[00:22:58] David Kaufer: Wow.

[00:22:59] Barry Prizant: So that's a huge number.

[00:23:02] David Kaufer: That is a huge number. And I as you're talking about it, I'm thinking, you know, as it relates to the autistic and the non speaking community, is that how frustrating that must be. So, you know, because there are a lot of non speakers who have gained communication through the letter border typing or you know other methodologies. That that's when it finally comes out that they have vision issues. It could be double vision or you know, so similar types of issues. Of course they had no way to share that before. Right? So they're like you said, they're trying to they're trying to cope they're trying to cope with vision issues without having the way to share that, to communicate, like, hey, I'm I can't see in three D or I can't, you know, I'm seeing double of every you know, these words are all of that.

[00:23:56] Barry Prizant: Yeah. Well and I mean everyone thinks that this is just the way you see. They have no idea that someone else sees differently than they do.

[00:24:05] David Kaufer: Right, Like to your point, like growing up, right, growing up, you're like, oh it's just the this is just the way it is.

[00:24:13] Barry Prizant: Yeah, my daughter had no idea that anyone saw any differently than she did either, you know.

[00:24:18] David Kaufer: And she didn't think it was and she didn't think it was I wanna say weird, you know, but different. She didn't think anything about reading the way that she wrote read, for example. Like that was just the way it was. I you just, you know, cover up one eye and roll over and do the other eye. And that's just the yeah, that's just what you do. Yep. So what role does neuroplasticity play in vision? I mean, what you know, when it comes to this, like you said, it's not just twenty you know, looking twenty Are there

[00:24:28] Barry Prizant: Hello. Exactly.

[00:24:47] David Kaufer: Are the ther is vision therapy, does that deal with issues with the brain in terms of training the brain as well?

[00:24:54] Barry Prizant: That's exactly what it's doing. Yeah. Okay. We're training the brain to see correctly. Cause the eyes themselves are just an organ, right? They the muscles are not gonna do anything you don't tell 'em to do. So if the brain isn't telling the eyes what to do, you're not gonna see properly. Regardless, you know, it real vision really is just in the brain and we don't think of it that way. We think it's what our eyes are seeing, you know. But I mean our arms aren't gonna move if our brain doesn't tell it to. So why do we think our eyes know what to do if our brain doesn't tell 'em what to do?

[00:25:31] David Kaufer: Why do we think that, Denise? That's a good question. I mean th that 'cause it's true. That's a absolutely the perception. You don't think of it as a muscle, you don't think of it as something that your brain has to control. I guess unless you're really in a situation when you that kind of focus is required. Anything from like looking through a microscope or binoculars or you know, whatever where you're like, Oh, you know, you're actually focused on the task at hand, I guess. You just take it for granted? I mean

[00:26:03] Barry Prizant: We all take it for granted all the time. And Yeah.

[00:26:08] David Kaufer: Do you Yeah. Go ahead. Kind of like walking. Again, you do you take advan you take it for granted if you're able to walk. Right. If you've gone through that progression, you know, that's all you know. You learn to crawl, you learn to walk. You want to go from point A to point B, you walk there. You don't think about your brains telling your legs to move. I guess it's the same for vision.

[00:26:31] Barry Prizant: Exactly. And if our eye muscles aren't working properly, it's no different than any other muscle in our body, you know, and we do exercise to strengthen those muscles or to relax those muscles, right? And in the case of our eyes, we really need to relax the muscles in order for them to see properly. And we never think of it that way either, you know.

[00:26:57] David Kaufer: No, not at all. I mean you're starting to see some awareness in terms of eyes. I'm thinking about like with eye fatigue now with all the screen time, you know, things kind of related to that. Hey, you need to give your eyes a break. You know, you don't want to be staring at a screen for hours on end and all that. But I still feel like it's fairly shallow in terms of the approach that we take.

[00:27:22] Barry Prizant: I agree. Absolutely. Yep.

[00:27:28] David Kaufer: yeah, I was thinking also with especially with kids and screen time and you know, this all overlaps, but could some you know, could these problems sometimes I look like ADHD or learning disabilities or sensory issues?

[00:27:43] Barry Prizant: Absolutely. Yeah. And the fact that we see it in this population should point that out a little bit. You know, if the same kids that are struggling are the ones who have a vision problem, then w you know, which came first? And what's the best way to deal with it, right? And you know, I have another favorite book that I wanted to share.

[00:28:10] David Kaufer: Yeah, I'm pleased.

[00:28:12] Barry Prizant: There's well, there's a couple that were written by Robin and Gillian Benoit. One of them's called Jillian's Story. And then I have this one as Dear Jillian. And they talk in this book about well, in the first one, she tells Jillian's story. And Jillian had amblyopia. she had passed every eye exam. And the way she passed it, and I don't know if you know what amblyopia is or if you're

[00:28:39] David Kaufer: I do not

[00:28:40] Barry Prizant: Okay. Well s it's a type of strabismus. Amblyopia you have lower acuity in one eye. So if you can't see very well out of one eye, you're just gonna use the other eye. And that eye just kind of shuts off. And it sometimes if that happens for a long period of time, the person really goes blind in that eye. And it just kinda becomes they treat it as a permanent kind of problem. I don't agree that any of that's permanent, but There's different perceptions on that. Anyway, Jillian had pretty significant amblyopia, but she'd passed every test when they've done the eye exam at the school because she thought it was like a memorization test. And so she would listen to the kids in front of her and she wanted to get the treat, right? And so she would just rattle off whatever they said, even though she couldn't see it at all.

[00:29:34] David Kaufer: Wow. That's I mean it's ingenious, but again not helpful in term

[00:29:40] Barry Prizant: Exactly. Yeah. And so when she finally got into vision therapy, then, you know, she was able to overcome that and gain 3D vision and then tell her story, you know. And then all these people started writing to her saying, Hey, let me tell you my story. And that's what went into the second book, right? And she did talk to someone that was autistic. Their family. Right.

[00:30:04] David Kaufer: Oh interesting.

[00:30:06] Barry Prizant: And that's why I wanted to share this because I think it this is a really great story about it. he this child was only two when they figured out that he well he was diagnosed as autistic, but then they figured out that he also had an eye issue that could be helped with vision therapy. So they took him to vision therapy and he was able to progress so much that he graduated from the autistic program that he was in with the school system. They said he doesn't qualify as being autistic anymore. So I think that that's just kind of points to the fact that there's so much more going on.

[00:30:40] David Kaufer: Interesting. Yeah. So much more going on. Absolutely.

[00:30:52] Barry Prizant: And we c we can we can look at the symptoms and say, well, it's gotta be this, but that may not be the case, you know. And so there's a lot of misdiagnosis of ADHD and dyslexia and all of those learning issues that we really should check the vision to see if that's what's causing the problem, or if it can help the problem. It's all working together.

[00:31:17] David Kaufer: Right, for sure. And it seems that there is not a big emphasis on vision screening, really. I mean it kind of it's almost like kind of a last resort. And do you think again, is that lack of awareness, Denise, or is it just resources? I mean what how what would you attribute or would you

[00:31:40] Barry Prizant: There's so much that goes into that's such a loaded question. I had I had to remove myself from the school system because it was such a hot topic for me personally. Because I wanted to go in there and I wanted to tell every parent, you know what, if your child is struggling in school, you need to take them to a developmental optometrist, not a regular optometrist. Right.

[00:31:44] David Kaufer: Okay, it's

[00:32:07] Barry Prizant: Those guys tell you everything's fine your whole life. That's what happened for me. That's what happened to my daughter. I specifically asked my optimetrist if she had a problem with depth perception. And he told me she was fine. So you can't even get this kind of a diagnosis at a regular doctor's office. And they're not gonna screen for it in a school system, right? They're only screening for acuity, you know, at the eye chart across the room. There are things they can do. that are not that hard, but they're not equipped all the time to do it. And what I found when I said, you know, I really want to tell this parent that they should take their child to their per correct eye doctor, the school system said, You can't tell them.

[00:32:53] David Kaufer: Why what? Why not?

[00:32:55] Barry Prizant: Because the school doesn't want to have to pay for vision therapy.

[00:32:59] David Kaufer: Yeah. Comes if it comes down to finances. So they don't they don't they aren't even questioning I'm drawing a parallel because in terms of like in the non speaking world the there are issues with schools not allowing the use of letterboards or communication partners in there. And again, part of it's financial, part of it in this case is also claims that, you know, it's not effective. I mean they there's a lot of other excuses they use in this. But I was curious, so with this though, you think it's purely financial are do they don't even they don't care if it's effective or not. They just care about the cost.

[00:33:42] Barry Prizant: Well, they're not trained to do it and they're they don't have the funds to pay f for, you know, a developmental optometrist to do it.

[00:33:51] David Kaufer: But they can still recommend it, couldn't they? I mean, couldn't they at least or educate I don't see the harm in at least I understand them saying, hey, we can't provide this necessarily if this is an additional cost, additional burden. I mean, I know financially, right? I mean, our we ask a lot of our schools and s and stuff, and that's a another topic, but I can at least understand that from a financial standpoint, if that's just an additional cost to burden. But to at least say, hey, here's something you should look at.

[00:34:22] Barry Prizant: I think it's a big problem. Yeah. That's why I'm saying I had to just not be in education. You know? And that kind of pointed me towards what I ended up doing, right? At the end of the day, that's why I'm doing podcasting because we need awareness.

[00:34:42] David Kaufer: So that really what that really did just kinda cause you were teaching, right? You were substantial.

[00:34:47] Barry Prizant: I was substituting yeah I it Yeah, no, in the meantime I had to find something that was gonna pay the bills. Right. So instead of going the route of alternative I route to licensure and getting a teaching certificate and being in the school system, I got my insurance license. I sold insurance for ten years. Finished raising my children and all of that. And in the meantime, you know, that gave me flexibility to focus on my own healing, to do my vision therapy, to get my child in. And you know, get all of that handled, get them all married off. And then that's good. then I was able to spend more time on my podcast.

[00:35:30] David Kaufer: Ha And how and remind me how long you've been doing your podcast?

[00:35:41] Barry Prizant: I started it in twenty about the time.

[00:35:45] David Kaufer: Wow, good for you. Wow. That's amazing.

[00:35:48] Barry Prizant: Yeah, well, and I started it before I had everybody out of the house and there were some downtimes. You know, I think every podcaster gets to the point where they're questioning whether they're able to continue, right? And my point in time happened when I had three daughters get married in the space of four months in the summer of twenty one. I blame that on COVID.

[00:36:15] David Kaufer: COVID marriages.

[00:36:16] Barry Prizant: Well, they were all dating from the dating apps and stuff, you know? And they all

[00:36:22] David Kaufer: Yeah, plan that a little bit, spread it out a little bit for your sake? I mean, geez, that's a lot.

[00:36:28] Barry Prizant: You would think. Yeah. So that happened and I had my third daughter get married by September of that year. And then the f and I had one podcast come out that year, you know, in that summer. Yeah. And then I had some technical issues and I just let life get to me, you know, the whole now I'm an empty nester, what's my life look like? And then in April of twenty two my I actually lost my husband in a motorcycle accident. Oh And so that kind of shut me down as a podcaster too, you know. So people look at my dates on my podcast, there's this huge gap in there, you know. Oh I finally had to just say, you know what, this is why I'm still here, part of why I'm still here, you know, is because I still have work to do. I mean, there's a reason why he had to go and I had to stay, you know. So I got back into it.

[00:37:09] David Kaufer: Well understandably. What are some of the lessons you've learned? I'm curious from podcasting, first of all, and some of the guests that you've had on there. What have what are or what are a couple of maybe surprising things that have come from it, that journey?

[00:37:45] Barry Prizant: Surprising is a good question. I started to see how the whole body fits into the picture. If we look at it just from the eye standpoint, we limit ourselves, you know? And if we look at it from a whole body perspective, then we can say, okay, what is the real cause of this problem? Is there an alignment problem in the body that we can look at?

[00:37:55] David Kaufer: Yes.

[00:38:15] Barry Prizant: that will help us along the way or that is signaling that there's something going on with the eyes even. I for this is a great example of my from my own life. I was born pigeon toed. So we already have a misalignment, right? The way they handled that back in my day, since I'm so old, is that they would put casts on your on the baby's feet.

[00:38:40] David Kaufer: I remember hearing about that.

[00:38:42] Barry Prizant: for like six weeks. And then after the cast comes off, then they put a bar with two with the orthopedic shoes, right? And the shoes are pointing out and you have to wear that and most of the time, you know. I remember it being during naps and stuff, you know. Like it was it was a thing that well, I had to do it with my oldest daughter too. Like I my daughter had the same problem. So I know. So there's that misalignment that I think you should pay attention to, you know, because it the body's telling us something, right? It's telling us that we need to pay attention to what's going on as far as oh I take it I personally I take it back to feelings too, but with my guests I have talked with them about the fact that there's misalignments in the body that we could address as well. So it's kind of a long answer. I don't know if that gives you

[00:39:45] David Kaufer: It does. Yeah. No, I no, I'm just kinda thinking as you're talking, about that. You know, the whole body approach. I th I think it makes so much sense. And I think it's something that we've continued to learn about, you know, as people in the medical profession. It's kind of getting back to what you said earlier. It's kind of funny. You can we start w the topic was talking about kind of the vision therapy and then of course You think about just the eyes, right? This is just a you know has to and we and we do that with our body, right? It's just and in healthcare in general. Just one thing. Oh, you have a problem with headaches or whatever. So it's just, you know, it's a it's a headache problem. And you know, where could we find out later it could be something related to a spine issue, you know, a misalignment that's everything's to your point. Everything is so much more connected than I think we're taught or realize or assume or whatever. I'm you know, I'm again I'm not sure. It's kind of like one of those systemic things. And I'm sure I think some of it's probably just kind of the you know, traditional healthcare approach that specialists in our age of specialists, you kind of choose especially and so right. I don't know. And they don't talk right.

[00:41:05] Barry Prizant: And they don't talk to each other. And in our world today with vision even, the ophthalmologists and the vision therapy doctors don't typically talk to each other either, which I think is malpractice, honestly, because there's so much good that can be done when they collaborate. You know, if I hadn't had vision therapy, surgery, and more vision therapy, I would not have gained three D vision.

[00:41:34] David Kaufer: Right.

[00:41:35] Barry Prizant: But there's so few doctors that work together. You know, and if you go in and you only have the surgery, the percentage of success isn't even being measured. If you want if you want the result to be three D vision, they're only measuring whether the eyes are aligned. So it th they consider it a 'cause cosmetic surgery.

[00:41:56] David Kaufer: Right. That makes sense. Wow. Why don't they talk more? Is it ego? Is it just is I mean, is there is it is it a turf battle? I mean, I honestly don't know. I mean again we see this in other areas, right? I mean I I'm drawing analogies to you know the world I'm most familiar with in the autism world, but in this world, what do you see, Denise?

[00:42:21] Barry Prizant: I think it's an ego problem personally, you know. I think they're so t caught up in where's the scientific proof, you know? Well who's gonna do these studies that prove that you're gaining three D vision, you know? It's

[00:42:40] David Kaufer: So that's is that kind of their

[00:42:43] Barry Prizant: Yeah, that they'll say there's no studies that prove it works. Well, there's all kinds of people who can say, look, this worked for me, you know, but everyone's problem is so specific. Doing double blind studies doesn't even work. I mean, there's how would you even do a study like that? You can't find people whose situation is exactly the same, no one's is. So you can't do a study on it.

[00:43:01] David Kaufer: How would we right. That is so interesting. and again, that I was just gonna say, Denise, that's the connection. That's exact because that's exactly the battle that parents and you know non speakers are fighting as well in our world. And when talking about the world of letterboards and you know, we call it spelling to communicate and all this, the establishment, in this case the speech and hearing association, therapist association. They use the exact same argument. There's no s you know, this isn't scientific. There's no scientific proof. And the response is, you know, the proof is in what you're seeing. I mean, people are using it successfully. And it sounds very similar to what you're saying with vision therapy. So it sounds like kind of traditional I eye doctors see vision therapy as I don't know, is do they see it as like woo-woo sort or kind of

[00:44:06] Barry Prizant: Yeah, I think they do. there's so many times that they just say, Oh, it doesn't work, or you're not a candidate, or they don't even mention it to someone who's been in for vis for eye surgery. They'll say, Well, this surgery might not work, you might need another one. I mean, I'm on these support groups for Strubismus on Facebook and it's a constant thing.

[00:44:32] David Kaufer: Wow.

[00:44:33] Barry Prizant: They just they don't tell him about it.

[00:44:36] David Kaufer: Yeah, that's like I said, that's that blows me away in a way, again the parallels that we see. And you don't you don't know what you don't know. So going back 'cause you know, one of the c obvious questions would be, Denise, is for parents, what should they be looking for? I mean, if your if your eye doctor's not gonna give you a heads up, like, hey, here's you know, here's something to look for, if they're not gonna steer you to vision therapy or you know, some of these other you know Treatments? What should the parents be looking for?

[00:45:09] Barry Prizant: Well, I kind of alluded to it a little bit with what I mentioned about my daughter's story. You know, th asking those specific questions if your child is verbal and can tell you, right? Obviously it's way harder if your child's not verbal and they can't tell you, but I have a little checklist that I made for parents. Again, it's gonna it's gonna be a little harder depending on your child. But if they're

[00:45:38] David Kaufer: But no, it's is it on your website or do you do you have that I want to hear it?

[00:45:41] Barry Prizant: Yeah, and I can go I can go over the short list. It's a very yeah, so they lose place or skip lines or reread something and when they're reading. they avoid reading or they get frustrated really quickly. they would understand better when they're listening than when they're reading. They have a short attention span for schoolwork, but if they have a preferred activity, their attention span is long. they get tired or emotional or overwhelmed during learning tasks.

[00:46:17] David Kaufer: Right.

[00:46:17] Barry Prizant: They're clumsy like I was. They bump into things or they struggle with coordination. anytime you've been told everything is fine and you still have concerns, that's the time to go and have it checked out, you know. But the thing is that a regular optometrist or an ophthalmologist is not gonna look for the same kinds of things as a developmental optometrist will look for.

[00:46:42] David Kaufer: Right.

[00:46:43] Barry Prizant: So you really have to go to the right doctor.

[00:46:47] David Kaufer: That makes a lot of sense now. And this is kind of this may be kind of a dumb question, but can if your child has, you know, this type of vision and w I mean, would they still be able to appear normal, yeah, but they have normal vision in other ways, like playing video games or things like that. Is that affected?

[00:47:09] Barry Prizant: Yeah, I mean a video game isn't gonna have a depth perception requirement.

[00:47:14] David Kaufer: So that's easy that's easier 'cause it's not okay.

[00:47:16] Barry Prizant: Yeah. I mean it's in two D most of the time, you know, unless you're playing with the VR headset. Which actually a lot of vision therapy doctors are starting to use the VR.

[00:47:31] David Kaufer: I was wondering about that, if that would be helpful with that.

[00:47:36] Barry Prizant: Yeah, I actually even have one and I I've been so anti-video games that I can't make myself do it, but I bought one and I was like, I'm gonna do this. And I haven't. But it is something that a lot of patients find very helpful.

[00:47:51] David Kaufer: Yeah, no, you've just you've got me thinking in terms of and I'm just thinking, are there are there any other questions that you can ask your child again that maybe aren't reading related or things I'm thinking about again, our audience who maybe have non-speakers but can communicate through typing. are there any other side things that they can ask or ways to try to determine if this is a problem or a potential problem or issue?

[00:48:17] Barry Prizant: This things I'm seeing in on these checklists are gonna have to do with being able to keep your place, you know. So the hand eye coordination piece could apply to nonverbal, you know, if they if they have a hard time even picking something up or catching or whatever, right? That's a high hand eye coordination issue. I remember Gillian saying that she used to stick out her pinky when she was grabbing the cup. You know, because it helped her orient where her hand was gonna go. oh You would see compensations in that hand eye coordination piece.

[00:48:48] David Kaufer: Where her hand was, yeah. Okay. Great. Well, let's move on to our weekly segments because I know that you are prepared. Thank you, Denise, for coming prepared. yeah. And so we're gonna start off and for each of these, just give you fair warning, we do have little jingles that will that Dave, producer Dave will play and then we'll get we'll start off with the tip of the week. It won't necessarily be autism tip of the week, but it'll be Denise's tip of the week.

[00:49:27] Barry Prizant: Hey parents, gather round, it's time to speak We're here with your tip of the week Strategies and wisdom just for you Autism parents, we know what you go through

[00:49:44] David Kaufer: All right, it's time for our tip of the week. And as I mentioned earlier, I know that Denise, you brought one for us. What do you have?

[00:49:53] Barry Prizant: Well, we've kind of talked about it a little bit already, but just pointing to the fact that a child may be able to see clearly and still have difficulty using their two eyes together, and a regular vision screening, even at a doctor's office, may not tell the whole story.

[00:50:11] David Kaufer: Okay, that's a great that's a great tip. and I was gonna ask kind of a related tip, if every parent could know one thing about binocular vision, what would it be?

[00:50:26] Barry Prizant: Well, not everyone has it. Okay. You know. Yeah. It I think we just take that for granted. not just as a parent, but as a as a person alive on the earth today, we just think everyone sees like we do.

[00:50:40] David Kaufer: Yeah, right. You just assume that.

[00:50:43] Barry Prizant: I've talked to so many people they don't even know what binocular vision means. They and I there I say, Well, I didn't see in three D before and they say, Well, do I see in three D? I said, Well, probably. I don't know. I can't see like you do.

[00:50:57] David Kaufer: I don't think. Yeah, that's funny. All right. Next up we have factor fiction. Time ready, and here we go. This is autism factor fiction. We're breaking down what's real, what's rumored, and what might surprise you. Let's go. All right. So this is again where I like to introduce potential stereotypes or myths or, you know, things that may misperspe misperceptions that may be out there. But

[00:51:07] Barry Prizant: oh

[00:51:30] David Kaufer: Denise, what do you have for fact or fiction?

[00:51:33] Barry Prizant: Well, again, we've talked about a lot of this, so I'm kind of feeling like maybe this is just a review. the fact or fiction that I chose was twenty means that your vision is fine.

[00:51:50] David Kaufer: Right. I think that's a big one though. I think that bears repeating because I think that's such a that it that I think that's just presumed to be fact. I think that's just so ingrained in us that that's the goal. It's just all about twenty vision that you don't even don't even think about that.

[00:52:09] Barry Prizant: Right. And because my gl my glasses corrected my vision so that it was twenty I must be fine. Right.

[00:52:18] David Kaufer: Yeah. And I guess kind of related to that would be, you know, I'd jotted down, you know, like my child passed their vision screening, you know, at school. So I'm everything's fine. That yeah. That true.

[00:52:32] Barry Prizant: And being told that you're fine for years and years and years. And knowing that you're not really.

[00:52:41] David Kaufer: I mean there's a little bit of trauma that goes with that, right? I mean kinda isn't it a little bit of being like gaslit for so many years? Like you're like, I know this isn't true, but I can't prove it.

[00:52:52] Barry Prizant: Yes, and I've heard it from many of my guests as well.

[00:52:57] David Kaufer: That they knew that they knew something was wrong, but

[00:53:01] Barry Prizant: Yeah, and one in particular had the strabismus surgery and went back to her surgeon and said, You know, I'm really struggling. She had a complete breakdown of her visual system after her surgery. You know, you move the eyes around, she's used to compensating a certain way. And her eyes had no idea what to do at that point. And this was a while after the surgery that she had this complete breakdown. And they treated her like she was crazy, like she should just But they literally said, it's all in your head.

[00:53:35] David Kaufer: Unbelievable.

[00:53:37] Barry Prizant: It was all into her head because it was her brain not knowing how to tell her how to work together. Right.

[00:53:43] David Kaufer: Right, No, I go back to li the point you made w right off the bat, Denise, in terms of how our eyes are muscles and if you're gonna have surgery on a muscle, it's just like anything else. Like I can't imagine someone who goes through, say, you know, again, sports related that people relate, you know, can go through ACL surgery and just like you just have surgery and then say, Okay, you're good to go. Without PT, right? Without any type of follow up. I mean, it sounds like that's kind of the expectation with eye the eye surgery.

[00:54:17] Barry Prizant: It is. Yeah. And that's actually one of my favorite analogies. You would never have surgery on any other part of your body and not have physical therapy surgery or after surgery or both. Right. And yet that's what we are doing all the time with vision with our eyes with our eye surgery.

[00:54:26] David Kaufer: Yeah. That's crazy. Sorry. That's that really is crazy. I mean, that's just I guess it just seems yeah. It seems like there's we need to progress the view there. Like in as we have in so many other areas of medicine and healthcare. I'll put it that way. Saying it's just crazy isn't very productive.

[00:54:58] Barry Prizant: Well, it's not but the a awareness is what we're trying to do so that people will know that there needs to be a ship. You know, it we ha we have to demand it of the doctors.

[00:55:08] David Kaufer: Right. Yeah. And that's yeah, and that that's kind of like a whole nother conversation, right, Denise, is changing our mindset. as patients in general, as advocates for our kids, you know, dealing with doctors. trusting them as experts, which they are, they're trained, they're, you know, they have a lot of education and training, which deserves respect. But that doesn't mean they're all known. all knowing and they and they don't know our bodies. I mean they can't at best a lot of what they're doing is still kind of making educated guesses about the approach, which, you know, that's just the way it works. But

[00:55:52] Barry Prizant: And it's true in the vision therapy room too. They have to figure out where the missing piece is, you know. And you can talk about vision therapy with relation to traumatic brain injury and everything too. And, you know, I've had many of my guests that work with that. population say if you've treated one person with a traumatic brain injury, you've treated one person with a traumatic brain injury, you know. So there's just a lot of things going on with that, you know, and I think some of the autistic population falls into that traumatic brain injury category too, right? And so it's just they're practicing medicine, you know, they're trying to figure it out.

[00:56:27] David Kaufer: Yeah. Yeah, for sure. No, I mean it's fascinating. All right. Well, next up on our segment is gonna be our confession of the week, parent confession.

[00:56:48] Barry Prizant: Teens can go astray, found socks in the fridge one day. Laughter's the best remedy, hey, we're just finding our own way. Sharing joys and moments we seek, honest tales that make us unique. Join the crowd, we're never meek.

[00:57:14] David Kaufer: All right, now's the time when we get to, as I always say, share our humanity so that we don't have just have the answers. We've made mistakes too. And Denise, I mean, with six kids, I'm sure there's a couple of things that may have come to mind. But what, yeah, what would you like to share with everyone?

[00:57:32] Barry Prizant: Well, you know, we I already talked about my daughter Abby and how I didn't know that she needed vision therapy till I knew, right? I don't talk about this a lot, but I actually had a another child who had some struggles and still does actually. there's another vision issue called convergence insufficiency. And My son had been struggling in school. We thought that he was just lazy because he could read, he could do the work. If he did the work, he got a hundred percent, he got an A. If most of the time he didn't do the work, most of the time. And it was not until he was 15 that we had him diagnosed.

[00:58:19] David Kaufer: Okay.

[00:58:29] Barry Prizant: And my doctor said it was probably the worst case of convergence insufficiency he'd ever seen.

[00:58:35] David Kaufer: What is it? Can you give it a

[00:58:37] Barry Prizant: Convergence insufficiency means that when you try to focus close, your eyes just give up. You can only sustain the convergence, the eyes working together close up for a limited amount of time before it's just too tiring and it it's

[00:58:56] David Kaufer: It's like a overload probably, right?

[00:58:58] Barry Prizant: And you just don't want to do it, right? Which is so why he wouldn't focus on his work, you know. Just didn't want to do it because it was so hard to do it. And I did I took him to vision therapy. He did several months of vision therapy, and he was just not really progressing with it, you know. Okay. Yeah. And so, you know, by that age, it's really hard to force them to do it.

[00:59:19] David Kaufer: Okay. Sure.

[00:59:29] Barry Prizant: And I gave up. That's my big convention actually. I don't I hardly ever talk about it.

[00:59:40] David Kaufer: Yeah.

[00:59:41] Barry Prizant: And it's still I mean, he doesn't if he wants to do something he figures out how to do it. He's very smart. He can do whatever he wants to. But it schoolwork is not it.

[00:59:52] David Kaufer: He's yeah, he it sounds like he's found other ways of coping and other ways to get things done. But he also knows that option's there, right?

[01:00:01] Barry Prizant: He does. Yeah. And I've told him that many times. You know, you can go back and do the vision therapy and get the results if you want to.

[01:00:12] David Kaufer: Yeah. Well that I can see where that would be really difficult. But there's only so much that you can do too, right? I mean, that's the part that I think we all struggle with as parents is we you know, we want to be able to control as much as we can. We want to be able to boost our kids and help them. And I was just having that conversation with my brother the other day. It's a it's a really tough balancing act between like is it pushing too hard or you know. and pulling back too far too. And I the unfortunately there's no there's no perfect answer, right? You just do what you just do what you think is best. But yeah, thank you. Thank you for sharing that. That's I mean I really appreciate that. Well we're gonna turn it around 180 and again because we like to end this on a lighter note. So we have our semi news segment which is light on the spectrum.

[01:00:52] Barry Prizant: Yeah. Light on the spectrum, oh a moment, person or story bring in something good. It feels hopeful, grounded, not performative. Use it for a person, a moment, a breakthrough, a story.

[01:01:31] David Kaufer: All right, so like I said, what do you have for us, Denise, to give our listeners, parents out there, a little bit of hope?

[01:01:40] Barry Prizant: Well, I think that the my story is my biggest gift, right? Because neuroplasticity is a real thing, you know. You don't have to say, Oh, this is just the way it is. And I mean I've heard it so many times. People will say, I have permanent double vision. I will never see in 3D. Happens all the time, you know. And When I went into my surgeon for my first visit with him and I told him that my goal was to see in 3D, he didn't take that super well, really. You know, he actually told me that he wanted me to come back in and do some more testing because they were getting some new machine in there that was gonna be able to gauge whether I'd be successful, that kind of thing. And I already knew I was gonna be successful. But He didn't, you know, and so I waited and I went back in and I did their all their s testing and stuff and found out that he actually and I'm it probably shouldn't share, but I knew he knew that he thought that I was being unrealistic.

[01:02:57] David Kaufer: Right, sure. He's being he's being overly cautious or overly skeptical or you know choose your choose your word, right? I mean.

[01:03:07] Barry Prizant: Right. But I mean neuroplasticity says our brains can change, right? The way we see can change, the way we do things can change, we our bodies can improve in many different ways, right? We see that all the time at every age. But yeah. Doesn't matter if you're fifty or seventy or twenty, right? It can change. And so my

[01:03:24] David Kaufer: We're out of

[01:03:36] Barry Prizant: gift is that I proven it can change. You know, I gained three D vision at age fifty four.

[01:03:43] David Kaufer: That's amazing. Yeah, that's a great message. That should give a lot of people hope. And like you said, especially the message about neuroplasticity and just how it can't be emphasized enough because again there's so many contexts where we hear that. like you said, I have permanent XYZ. for those of us like in an autism world, you know, we've dealt with a lot of negativity and we've and now we're seeing a lot of positives. So before we wrap up, Denise, how can people find you?

[01:04:16] Barry Prizant: Well, I have my podcast on every podcast player. And recently I put it on YouTube. It's got a static image. I don't do video because I'm not that but well I just haven't felt like I could edit a video. I'm good at editing audio. So it but it is on YouTube, it's on all the podcast players, Healing Our Site. I have a Facebook group also called Healing Our Site.

[01:04:26] David Kaufer: Okay.

[01:04:45] Barry Prizant: And website. Healing our site. Awesome. Yeah. So they can reach me in any of those ways.

[01:04:54] David Kaufer: Great. Well thank you, Denise, for joining us today. I really enjoyed the conversation. I know that our listeners and viewers will walk away looking at a vision. And eyes, maybe even the brain, a little differently. And thanks to all of you for spending part of your day with us. If you enjoyed today's episode, please subscribe, leave us a review, share it with someone you think would benefit from the conversation. This is one of the best ways to help us reach more families. Continue bringing these important stories to a wider audience. I'd also like to give a special thank you. To our producer, my guy, Dave Yaz, and the entire team at Pod 617. Dave's been with me since the beginning of our podcast. We're coming up on two years. His expertise, creativity, and friendship have helped make the lighter side of the spectrum what it is today. So again, thank you, Denise, and we'll catch everyone else next week.